



Health Psychology

Seminar 5:

The role of health psychology
supporting healthcare professionals

Overview of module

Unit	Topic	Assessment
1	Introduction to Health Psychology	
2	Understanding and Predicting Health Behaviours	
3	Health Promotion Interventions	CLQ poster and summary Peer responses Deadline:
4	Individual Differences	
5	Stress, Health and Coping	
6	Chronic Illness: Help-Seeking, Diagnosis, Adjustment	
7	Illness and Treatment Beliefs	
8	Psychological Processes at the End of Life	
9	The role of Psychology in Supporting Healthcare Professionals	Essay assignment: 2200 words: Deadline:



Recap Seminar 4

Illness cognitions

Self-regulation model

Research and support for the model

Psychological processes at the end of life



Aims of the seminar

This week's seminar will focus on the role of health psychology in supporting healthcare professionals. We will explore the incidence of burnout and compassion fatigue with healthcare workers and look at helpful strategies. We will also consider the importance of clear and empathetic inter-and-intra communication in healthcare settings.

You should prepare in advance for these discussions using information from the core textbooks and recommended reading.

This week we will **test your knowledge on:**

1. Barriers to effective communication in healthcare settings.
2. Compassion fatigue in healthcare professionals.



Question 1

What are the barriers to successful communication in healthcare settings?



Communication in health psychology

Ineffective communication can lead to poor health outcomes for both patients and staff working in healthcare settings.

What makes a good medical consultation?





Communication in healthcare settings

What are the barriers to poor communication? Things to consider:

Patient:

Professional:

Illness perceptions

Past experience

Health literacy understanding

Health anxiety

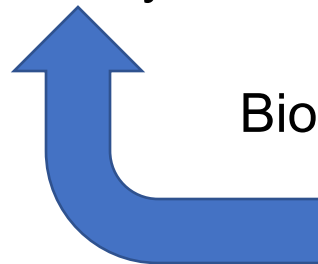
Health Care

Confidence issues

Lack of knowledge

Complexity of health issue

Lack of structure

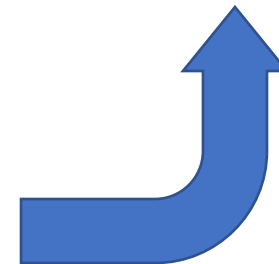


Biographical considerations

Power dynamics

Personality

Mood





Research

Nurse Education in Practice 19 (2016) 36–40

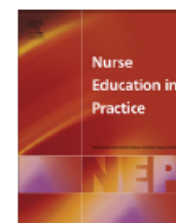


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Review

Interprofessional communication in healthcare: An integrative review



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ABSTRACT

The link between miscommunication and poor patient outcomes has been well documented. To understand the current state of knowledge regarding interprofessional communication, an integrative review was performed. The review suggested that nurses and physicians are trained differently and they exhibit differences in communication styles. The distinct frustrations that nurses and physicians expressed with each other were discussed. Egos, lack of confidence, lack of organization and structural hierarchies hindered relationships and communications. Research suggested that training programs with the use of standardized tools and simulation are effective in improving interprofessional communication skills. Recommendations include education beyond communication techniques to address the broader

What makes a good consultation?

10 things people want from their healthcare providers

- | | |
|----|--|
| 1 | To have a doctor who listens well |
| 2 | To have a doctor who is caring and compassionate |
| 3 | To have a doctor who explains information well and avoids medical jargon |
| 4 | To have convenient and quick access to healthcare |
| 5 | To have a doctor who treats them with dignity and respect, and as a partner in decision making |
| 6 | To have a doctor who is willing to spend enough time with them |
| 7 | To have a doctor who maintains eye contact |
| 8 | To have a doctor who asks good questions in order to understand their needs |
| 9 | To have a doctor who provides options and explains each option |
| 10 | To have a doctor who does his/her homework and knows the patient history |

Information from: Wen, L. S., and Tucker, S. (2015). What do people want from their health care? A qualitative study. *Journal of Participatory Medicine*, 7, e10; NRC Health (2016). What do patients really want from their physician?

Supporting our healthcare professionals

“The health and well-being of health care workers are essential to the quality of care they can provide for people and communities, affecting their compassion, professionalism and effectiveness”





Why do you think health care staff
are vulnerable to stress?



Research

Working in helping professions is emotionally demanding –
potential for compassion fatigue (Grant & Kinman, 2014; Kemper et al., 2015)

Why? Managing complex situations, uncertainty, lack of control, ongoing emotional interactions which provoke profound emotional reactions

Reflective ability and empathy associated with emotional resilience and improved practice in helping professions (Mann et al., 2009; Grant & Kinman, 2014)



Compassion fatigue

- Compassion fatigue can develop when the caregiver becomes increasingly involved and exposed to another's suffering, trauma and continuous life demands (Coetzee & Klopper, 2010).
- This then can result in a reduced ability to continue empathising (Day, Anderson & Davis, 2014).
- 40% of healthcare professionals report suffering from compassion fatigue (van Mol et al, 2015).
- Although commonly associated with professional carers, a more recent body of research highlights its relevance to the informal (or family carer) as well.



Impact of pandemic on nursing staff

Two thirds of nurses feel their mental health has deteriorated since pandemic (Spring, 2020).

Nine in ten nurses feeling more stressed/anxious.

Moral suffering = moral injury, moral distress, compassion fatigue, burnout.



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Any questions?



Question 2

How can health psychologists help prevent compassion fatigue in healthcare professionals?



Some practical strategies

- Shared decision-making
- Rotating roles, so people at more intense roles
- Psychologically safe workplaces
- Emotional support: end of shift check-ins with focus on psychological health.
- Sufficient equipment & supervision.

- Recognising the early warning signs.
- Acknowledging the need for self-care.
- Boundaries
- Having interests outside of work
- Supportive work practises.



How can health psychology help?

By exploring the binary concepts of health and illness and its relationship to the caring professions;

By revisiting the theoretical models and research;

By promoting the biopsychosocial model;

By cultivating attitudes and values

By empowering staff to develop knowledge, skills and confidence

By changing behaviour!



What do we mean by intervention?

Psychological interventions = actions performed to bring about change

Individual or group therapy

Family/Couples therapy

Psycho-education i.e., workshop, training

Motivational Interviewing

Rehabilitation work



Application of health psychology

Example 1:

Developing a training tool to develop person-centred care for careworkers

Example 2:

Psychological Boost workshops for nurses and midwives



Example 1: training tool for careworkers

What were the outcomes for the training?

Primary outcomes:

- Improving person-centred care skills in front-line staff

- Promoting psychological resilience

Secondary outcomes:

- Better care for older people in residential care settings

- Improved quality of life for older people in residential care settings



Preparation: Assessment - Formulation

Working groups

Assessment of needs – gap in the current training provision

Formulation: practicing and understanding empathy and resilience

Peer support

Reflective practice

Evaluation

Implementation

Development of training packs using behaviour change techniques:

- Action planning – workbooks where staff asked to write down how they would work differently
- Social support – the tool has built in sections for discussion/peer support
- Shaping knowledge – looking at distinctions between empathy and sympathy and eight person-centered care values

Michie, Atkins & West (2014)



Example 2: workshops for nurses and midwives

Post pandemic psychological boost training which aims to:

- Improve understanding of psychological stress and suffering
- Provide 'tools' to develop coping/resilience skills
- Provide a safe space to reflect and share experiences

Intervention:

Integrated therapeutic models (ACT, CBT, CFT) guided workshop



Implementation

- Development of 3 hour workshops guided by therapeutic models:
 - Improve knowledge of moral suffering and compassion fatigue
 - Introduce self-compassion exercises
 - Incorporate reflective practice
 - Provide workbook and resources
-
- Evaluated by pre and post questionnaires



Why measure outcomes?

- Impact of intervention
- Test formulation
- Goal setting
- Progress / motivation
- Better outcomes
- Evidence base
- Reflective practice





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Any questions?



Check your understanding

Question 1:

What are the barriers to successful communication in healthcare settings?

Patient:

Professional:

Illness perceptions

Past experience

Health literacy understanding

Health anxiety

Health Care

Confidence issues

Lack of knowledge

Complexity of health issue

Lack of structure

Power dynamics, mood, personality differences



Check your understanding

Question 2:

How can we help prevent compassion fatigue in healthcare professionals?

Education regarding susceptibility

Promoting psychological safe workplaces

Encouraging reflexivity

Promoting strategies:

Time management, boundaries, self-compassion



References

- Coetzee, S.K., & Klopper, H.C. (2010). Compassion fatigue within nursing practice: a concept analysis. *Nursing Health Science*, 12(2):235-43.
- Day, J. R., Anderson, R. A. and Davis, L. L. (2014). Compassion fatigue in adult daughter caregivers of a parent with dementia. *Issues in Mental Health Nursing*, 35, 796–804.
- Foronda, C., MacWilliams, B., & McArthur, E. (2016). Interprofessional communication in healthcare: An integrative review. *Nurse education in practice*, 19, 36-40.
- Grant, L., & Kinman, G. (2014). Emotional resilience in the helping professions and how it can be enhanced. *Health and social care education*, 3(1), 23-34.
- Kemper, K. J., Mo, X., & Khayat, R. (2015). Are mindfulness and self-compassion associated with sleep and resilience in health professionals?. *The Journal of Alternative and Complementary Medicine*, 21(8), 496-503.
- Mann, K., Gordon, J., & MacLeod, A. (2009). Reflection and reflective practice in health professions education: a systematic review. *Advances in health sciences education*, 14, 595-621.
- Michie, S., Atkins, L., & West, R. (2014). The behaviour change wheel. *A guide to designing interventions*, 1, 1003-1010.
- Wen, L. S., & Tucker, S. (2015). What do people want from their health care. *A qualitative study. J Participat Med*, 7, e10.
- West, M., Bailey, S., & Williams, E. (2020). The courage of compassion. *Supporting Nurses and Midwives to Deliver High-Quality Care. London: The King's Fund, London.*



Discuss and Debate

(a) What strategies can healthcare professionals adopt to promote psychological safety in their teams?



Discuss and Debate

(b) What are the implications of whistleblowing for healthcare professionals and how can they be supported?



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Any questions?